

Abiraterone

Abiraterone is a cancer drug. It is used to treat prostate cancer.

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What is abiraterone?

Abiraterone is a hormonal therapy drug. It is used to treat [prostate cancer](#).

It is best to read this information with our general information about [hormonal therapy](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How abiraterone works

Hormones are chemicals that our bodies make. They act as messengers and help control how cells and organs work. Hormonal therapy drugs change how hormones are made or work in the body.

[Prostate cancer](#) needs a hormone called testosterone to grow. Testosterone is mainly made by the testicles. Small amounts are made by the adrenal glands, above each kidney, and in the body fat. Abiraterone reduces the amount of testosterone made by the body. This may slow down the growth of the cancer cells.

Having abiraterone

You have abiraterone as tablets you can take at home. You have abiraterone with a [steroid tablets](#) called prednisolone to help reduce some side effects.

During your treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

Taking abiraterone tablets

Your cancer team will discuss your treatment plan with you.

Always take the tablets exactly as they tell you to. This is important to make sure they work as well as possible for you. Make sure you:

- do not take abiraterone tablets with food
- take abiraterone tablets at least 1 hour before food or at least 2 hours after eating food
- swallow them whole with a glass of water
- do not chew, open, break or crush them
- take them at the same time every day.

If you forget to take the abiraterone or steroid tablet, take your usual dose the following day. Do not take a double dose. If you have missed more than 1 dose, tell your doctor or nurse straight away.

Other things to remember about your tablets:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.

- Get a new prescription before you run out, and make sure you have plenty for holidays.
- Do not throw away unused tablets. Return them to a pharmacy.

Your cancer team may also give you other medicines to take home. Take all your medicines exactly as they tell you to. Do not stop taking any of your medicines unless your doctor tells you to.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

When you have this treatment, you will have regular appointments with a doctor, nurse or pharmacist. Always tell them about any side effects you have. They can give you:

- drugs to help control most side effects
- advice about managing side effects.

Most side effects can be managed. But sometimes side effects are harder to control. It is important not to stop taking hormonal therapy without telling your doctor. If side effects cannot be managed, your doctor may suggest a different type of hormonal therapy.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Fluid build-up

This treatment can cause a build-up of fluid in the body. This will slowly get better after treatment ends. Tell your doctor, nurse or pharmacist if you:

- are gaining weight
- become breathless
- have swelling in your face, legs or ankles.

They can give you advice and treatment to help.

Low levels of potassium in the blood

This treatment can cause low levels of potassium in the blood. You will have regular blood tests to check your potassium levels.

If your levels are very low, you may need treatment and your doctor may tell you to stop taking abiraterone. Symptoms of low potassium include:

- weakness
- twitching muscles
- feeling that your heart is beating harder or faster than usual.

If you notice these symptoms, tell your doctor, nurse or pharmacist straight away.

High blood pressure

This treatment can cause high blood pressure. Tell your doctor, nurse or pharmacist if you have ever had any problems with your blood pressure. You will have regular blood pressure checks when taking this drug. Let your doctor, nurse or pharmacist know if you have any headaches or blurred vision.

Risk of infection

This treatment can make you more likely to get an [infection](#). The most common type of infection when you are taking this drug is a urine infection. Some people can develop a serious and potentially life-threatening complication of an infection (sepsis) and need urgent treatment in hospital. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- urine (pee) that is cloudy or strong smelling, or that contains blood
- passing urine more often than usual
- pain or discomfort when you pass urine
- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- feeling shivery and shaking
- sore throat or cough
- diarrhoea
- breathlessness

It is important to follow any specific advice your cancer treatment team gives you.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Effects on the heart

This treatment might increase certain risk factors for heart problems. Tell your doctor if you have:

- had any problems with your heart
- high blood pressure
- high levels of cholesterol – a fatty substance in your blood.

They can monitor or treat these if needed. Talk to your doctor if you are worried about this.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during treatment:

- breathlessness
- dizziness
- changes to your heart beat
- swollen feet and ankles

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call 999 if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest

- difficulty breathing.

Raised level of fats in the blood

This treatment can raise the level of triglycerides. These are fatty substances in the blood. You may have a [blood test](#) to check the level of triglycerides. Your doctor may refer you to a dietitian for advice on diet or prescribe medicines to control your levels.

Bone thinning (osteoporosis)

Taking this treatment for several months or more can increase the risk of [bone thinning](#). This is called osteoporosis. It can make you more likely to get a broken bone (fracture).

You may have bone density scans to check your bone health before and during treatment. These scans can diagnose osteoporosis or osteopenia (low bone density) so that it can be monitored. If your scan shows osteoporosis, your doctor usually prescribes:

- drugs to protect your bones called [bisphosphonates](#)
- calcium and vitamin D supplements.

Regular exercise where you support your body weight, like walking, running and dancing, can help look after your bones. This is called [weight-bearing exercise](#). Other types of strength exercises are also helpful. If you have any bone thinning or the cancer is affecting your bones, get advice from your cancer team first.

Eating a healthy diet and not smoking also helps take care of your bones.

Skin changes

This treatment can [affect your skin](#). It may cause a rash, which may be itchy. Your doctor or nurse can tell you what to expect. If your skin feels dry, try using soap-free cleansers and unperfumed moisturising cream every day.

Always tell your doctor or nurse about any skin changes. They can give you advice and may prescribe creams or medicines to help. Changes to your skin usually improve when treatment ends.

Indigestion

This treatment may cause indigestion. Tell your doctor, nurse or pharmacist if this is a problem for you. They may be able to give you medication to help with this.

Muscle or joint pain

You may get pain in your muscles or joints. If this happens, tell your doctor or nurse. They can give you advice and painkillers or other drugs to help. Being physically active and keeping to a healthy weight may help with the pain. It can keep your joints flexible and reduce stress on them. If the pain does not get better, tell your doctor or nurse. They may talk to you about changing to a different hormonal drug.

Blood in the urine

This treatment can cause blood in the urine (pee). Tell your doctor or nurse if you notice this.

Adrenal gland problems

This treatment can cause problems with the adrenal glands, but this is not common. The adrenal glands sit at the top of the kidneys and produce hormones. If they do not produce enough of certain hormones, you may:

- feel tired and exhausted
- feel dizzy and light-headed when standing up
- crave salty food.

Other important information

Lactose

This treatment may contain lactose. If you have a lactose allergy or intolerance, ask your doctor, nurse or pharmacist for more information.

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Contraception

Your doctor, nurse or pharmacist will advise you not to [make someone pregnant](#) while having this treatment. The drug may harm the developing baby. It is important to use effective contraception during treatment and for a while after it finishes.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help